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CIN : U74140DL2015PTC279229

REGISTERED OFFICE:
LEVEL 3, Vasant Square Mall, Pocket V, Sector B, Vasant Kunj New Delhi - 110070

MC - 4271

PO No :PO1759747850-396

Name	: Ms.LALITA BHANDARKAR	Client Name	: TATA 1MG PUNE
Age/Gender	: 66/Female DOB:	Registration Date	: 17-Apr-23 10:26 AM
Patient ID	: MGP240740	Collection Date	: 17/Apr/2023 08:51AM
Barcode ID/Order ID	: D2918006 / 7076034	Sample Receive Date	: 17/Apr/2023 11:17AM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: WHOLE BLOOD-EDTA	Report Date	: 17/Apr/2023 12:34PM

HAEMATOLOGY

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Glycosylated Hemoglobin (HbA1c)	**7.3**	%	4 - 5.6	HPLC
Estimated average glucose (eAG)	**162.81**	mg/dL		Calculated

Comment:

Interpretation: HbA1c%

≤5.6	Normal
5.7-6.4	At Risk For Diabetes
≥6.5	Diabetes

Adapted from American Diabetes Association.

Comments:
A 3 to 6 monthly monitoring is recommended in diabetics. People with diabetes should get the test done more often if their blood sugar stays too high or if their healthcare provider makes any change in the treatment plan. HbA1c concentration represent the integrated values for blood glucose over the preceding 8-12 weeks and is not affected by daily glucose fluctuation, exercise & recent food intake.
Please note, Glycemic goal should be individualized based on duration of diabetes, age/life expectancy, comorbid conditions, known CVD or advanced microvascular complications, hypoglycemia unawareness, and individual patient considerations.

Factors that interfere with HbA1c Measurement: Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified

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Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Whole Blood-EDTA	Report Date	: 17/Apr/2023 12:34PM

HAEMATOLOGY

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Complete Blood Count				
Hemoglobin	12.6	g/dL	12.0 - 15.0	Cynide-free SLS-HB
RBC	4.21	mili/cu.mm	3.8-4.8	DC Impedence method
HCT	39.5	%	36-46	Calculated
MCV	93.8	fL	83 - 101	Calculated
MCH	29.9	pg	27 - 32	Calculated
MCHC	31.9	g/dL	31.5 - 34.5	Calculated
RDW-CV	13.6	%	11.5-14	Calculated
Total Leucocyte Count	7.77	10^3/µL	4 - 10	Flowcytometery/Microscopic
Differential Leucocyte Count				
Neutrophils	45.0	%	40 - 80	Flowcytometery/Microscopic
Lymphocytes	37.0	%	20 - 40	Flowcytometery/Microscopic
Monocytes	**11.0**	%	2 - 10	Flowcytometery/Microscopic
Eosinophils	**7.0**	%	1 - 6	Flowcytometery/Microscopic
Basophils	0.0	%	0 - 2	Flowcytometery/Microscopic
Absolute Leucocyte Count				

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HAEMATOLOGY

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method

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Referred By	: Dr.	Report Status	: Final Report
Sample Type	: EDTA	Report Date	: 17/Apr/2023 01:32PM

HAEMATOLOGY

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Erythrocyte Sedimentation Rate	**33**	mm/hr	0-20	Modified Westergren

Comment:

- ESR provides an index of progress of the disease and is widely used as an indicator of inflammation, infection, trauma, or malignant diseases. Changes are more significant than a single abnormal test
- It is specifically indicated to monitor the course or response to the treatment of diseases like rheumatoid arthritis, tuberculosis bacterial endocarditis ,acute rheumatic fever ,Hodgkins disease,temporal arthritis , and systemic lupus erythematosis; and to diagnose and monitor giant cell arteritis and polymyalgia rheumatica.
- An elevated ESR may also be associated with many other conditions, including autoimmune disease, anemia, infection,malignancy,pregnancy, multiple myeloma, menstruation, and hypothyroidism.
- Although a normal ESR cannot be taken to exclude the presence of organic disease, its rate is dependent on various physiologic and pathologic factors.
- The most important component influencing ESR is the composition of plasma. High level of C-Reactive Protein, fibrinogen, haptoglobin, alpha-1antitrypsin, ceruloplasmin and immunoglobulins causes the elevation of Erythrocyte Sedimentation Rate.
- Drugs that may cause increase ESR levels include: dextran, methyldopa, oral contraceptives, penicillamine, procainamide,

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Barcode ID/Order ID	: D2918005 / 7076034	Sample Receive Date	: 17/Apr/2023 11:17AM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum	Report Date	: 17/Apr/2023 02:03PM

BIOCHEMISTRY

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Calcium	9.3	mg/dL	8.8 - 10	Arsenazo III

Comment:

Increased in: Hyperparathyroidism primary and secondary, Acute and chronic renal failure, Following renal transplantation, Osteomalacia with malabsorption, Acute osteoprosis, Malignant tumours (specially of breast, lung and kidney), Drugs: Vit. D and A intoxication, Diuretics, estrogen, androgen, tamoxifen, lithium

Decreased in: Hypoparathyroidism, Surgical and Idiopathic, Pseudohypoparathyroidism, Chronic renal disease with uremia and phophate retention, Malabsorption of Calcium and Vit.D, obstructive jaundice, Bone Disease (Osteomalacia and rickets), Drugs: Cancer chemotherapy drugs, calcitonin, loop-actives diuretics, Hypomagnesemia,Hypoalbuminemia

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Barcode ID/Order ID	: D2918007 / 7076034	Sample Receive Date	: 17/Apr/2023 11:17AM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: FLUORIDE PLASMA	Report Date	: 17/Apr/2023 01:17PM

BIOCHEMISTRY

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Glucose - Fasting				
Glucose - Fasting	**106**	mg/dL	70-99	Hexokinase/ G-6-PDH

Fasting Plasma Glucose (mg/dL)	2 hr plasma Glucose (mg/dL)	Diagnosis
99 or below	139 or below	Normal
100 to 125	140 to 199	Pre-Diabetes (IGT)
126 or above	200 or above	Diabetes

Reference : American Diabetes Association

Comment:

Impaired glucose tolerance (IGT) fasting, means a person has an increased risk of developing type 2 diabetes but does not have it yet. A level of 126 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes.
IGT (2 hrs Post meal), means a person has an increased risk of developing type 2 diabetes but does not have it yet. A 2-hour glucose level of 200 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes

Plasma Glucose Goals	For people with Diabetes
Before meal	70-130 mg/dL

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Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum	Report Date	: 17/Apr/2023 02:03PM

BIOCHEMISTRY

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Iron Studies, Basic				
Iron Serum	85	µg/dL	50-170	Ferene
Unsaturated Iron Binding Capacity	**359**	ug/dL	70 - 310	Ferene
Total Iron Binding Capacity (TIBC)	**444**	µg/dL	228 - 428	Caculated
Transferrin Saturation	19.16	%	16 - 50	Calculated

Comment:

Iron is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron is seen in iron deficiency and anaemia of chronic disorders.
Increased iron concentration are seen in hemolytic anaemias, hemochromatosis and acute liver disease. Serum Iron alone is unreliable due to considerable physiologic diurnal variation in the results with highest values in the morning and lowest values in the evening as well as variation in response to iron therapy .

Total Iron Binding capacity (TIBC) is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. Increased levels of TIBC suggest that total iron body stores are low, increased concentration may be the sign of Iron deficiency anaemia, polycythemia vera ,and may occur during the third trimester of pregnancy. Decreased levels may be seen in hemolytic anaemia, hemochromatosis, chronic liver disease, hypoproteinemia ,malnutrition.

Unsaturated Iron Binding Capacity (UIBC) is increased in low iron state and decreased in high iron concentration such as

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BIOCHEMISTRY

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Lipid Profile				
Cholesterol - Total	191	mg/dL	Age>20 years : Desirable <200, Border Line 200 to 239, High >240,	Enzymatic
Triglycerides	74	mg/dL	Normal: <150, Borderline: 150 - 199, High:200-499, Very High>=500	Glycerol Phosphate Oxidase
Cholesterol - HDL	47	mg/dl	40 - 60	Accelerator Selective Detergent
Cholesterol - LDL	**130**	mg/dl	Desirable: <100 Above desirable: 100 - 129 Borderline high : 130 - 159 High : 160 - 189	Calculated

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BIOCHEMISTRY

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method

- Indians are at a high risk of developing atherosclerotic cardiovascular disease (ASCVD); at a much earlier age; more severe in nature and have high mortality.
- Major risk factors have been found to be dyslipidemia (abnormal lipid profile), smoking, sedentary lifestyle, obesity, hypertension and diabetes. Dyslipidemia is most important and found to be very high in Indians (79%); hence control of dyslipidemia is the key healthcare target.
- **LDL-Cholesterol** (LDL-C) contributes most significantly to atherosclerosis and is the **primary target** of treatment.
- **Triglyceride** (TG) rich lipoprotein remnants also play a major role in CVD. Indians have higher triglyceride levels and lower **HDL-C** (good cholesterol) combined with increased proportion of small dense LDL-C; this pattern is called atherogenic dyslipidemia and is associated with diabetes, metabolic syndrome and insulin resistance.
- **Non-HDL-Cholesterol** (Non-HDLC) measures all atherogenic lipoproteins (LDL-C, VLDL, Lp(a), Apo-B). Monitoring of Non-HDLC is the **co-primary target** and is especially important in patients with elevated TG (e.g. diabetics, obese persons, metabolic syndrome) and those on statin therapy.
- **Lipid Association of India (LAI) recommendations (2020) -**
  - Screening of all Indians above the age of 20 years for CVD risk factors esp. lipid profile.
  - Risk factors known to promote atherosclerosis include: Age- male ≥45 years, female ≥55 years; Family h/o premature CAD (male <55 years, female <65 years), Smoking/tobacco use, Systemic hypertension, Low HDL (males <40 mg/dl and females <50 mg/dl.
  - Fasting lipid profile is not mandatory. Both fasting and non-fasting lipid profiles are important for managing Indian patients with dyslipidemia. Non-HDLC should be calculated in every subject.

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Sample Type	: Serum	Report Date	: 17/Apr/2023 02:03PM

BIOCHEMISTRY

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Liver Function Test				
Bilirubin-Total	0.70	mg/dL	0.2-1.2	Diazonium Salt
Bilirubin-Direct	0.27		0.0-0.5	Diazo Reaction
Bilirubin-Indirect	0.43	mg/dL	0 - 0.8	Calculated
Protein, Total	7.48	g/dL	6.4 - 8.3	Biuret, End Point
Albumin	4.11	g/dL	3.2-4.6	Bromcresol Green
Globulin	3.4	g/dl	1.8 - 3.6	Calculated
A/G Ratio	1.22	Ratio		Calculated
Aspartate Transaminase (SGOT)	27	U/L	5 - 34	NADH (Without P-5-P)
Alanine Transaminase (SGPT)	24	U/L	0 - 55	NADH (Without P-5-P)
SGOT/SGPT	1.09	Ratio		Calculated
Alkaline Phosphatase	64	U/L	40-150	Para-Nitrophenyl Phosphate
Gamma Glutamyltransferase (GGT)	**59**	U/L	9 - 36	L-G-G-3-C-4-N Substrate

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MC - 4271

PO No :PO1759747850-396

Name	: Ms.LALITA BHANDARKAR	Client Name	: TATA 1MG PUNE
Age/Gender	: 66/Female DOB:	Registration Date	: 17-Apr-23 10:26 AM
Patient ID	: MGP240740	Collection Date	: 17/Apr/2023 08:51AM
Barcode ID/Order ID	: D2918005 / 7076034	Sample Receive Date	: 17/Apr/2023 11:17AM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum	Report Date	: 17/Apr/2023 02:03PM

BIOCHEMISTRY

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method

concentrations up to eight times the upper reference limit.Slight to moderate AST elevations are noted in hemolytic disease.
•GGT is a sensitive indicator of the presence of hepatobiliary disease, being elevated in most subjects with liver disease regardless of cause. Increased concentrations of the enzyme are also found in serum of subjects receiving anticonvulsant drugs, such as phenytoin and phenobarbital.

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BIOCHEMISTRY

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Kidney Function Test.				
Blood Urea Nitrogen	8	mg/dL	8-23	Urease
Urea	18.15	mg/dl	17.5 – 49.22	Calculated
Creatinine	0.82	mg/dl	0.5-1.2	Kinetic Alkaline Picrate
Uric Acid	5.3	mg/dL	2.5 - 6.2	Uricase
Sodium	137	mmol/L	136 - 145	Indirect ISE
Potassium	4.56	mmol/L	3.5-5.1	Indirect ISE
Chloride	102.8	mmol/L	98 - 107	Indirect
BUN/Creatinine Ratio	10.3	Ratio	12:1 - 20:1	Calculated

Comment:

BUN is directly related to protein intake and nitrogen metabolism and inversely related to the rate of excretion of urea.Blood urea nitrogen (BUN) levels reflect the balance between the production and excretion of urea. Increased levels are seen in renal failure (acute or chronic), urinary tract obstruction, dehydration, shock, burns, CHF, GI bleeding, nephrotoxic drugs. Decreased levels are seen in hepatic failure, nephrotic syndrome, cachexia (low-protein and high-carbohydrate diets).
Urea is a non-proteinous nitrogen compound formed in the liver from ammonia as an end product of protein metabolism. Urea diffuses freely into extracellular and intracellular fluid and is ultimately excreted by the kidneys. Increased levels are found in

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Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum	Report Date	: 17/Apr/2023 03:14PM

Immunology

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Thyroid Profile				
T3, Total	0.86	ng/ml	0.35 - 1.93	CMIA
T4, Total	11.2	µg/dL	4.87 - 11.72	CMIA
Thyroid Stimulating Hormone - Ultra Sensitive	0.497	uIU/ml	0.35 – 4.94	CMIA

Comment:

- Below mentioned are the guidelines for pregnancy related reference ranges for TSH, total T3 & Total T4.

Pregnancy			
	TSH (µIU/mL) (as per American Thyroid Association)	Total T3 (ng/mL)	Total T4(µg/dL)
1st trimester	0.1-2.5	0.81-1.90	7.33-14.8
2nd trimester	0.2-3.0	1.00-2.60	7.93-16.1
3rd trimester	0.3-3.0	1.00-2.60	6.95-15.7

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Sample Type	: Serum	Report Date	: 17/Apr/2023 03:14PM

Immunology

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
High	Normal	Normal	Subclinical Hypothyroidism	
Low	Normal	Normal	Subclinical Hyperthyroidism	
High	High	High	Secondary Hyperthyroidism	
Low	High/Normal	High/Normal	Hyperthyroidism	
Low	Low	Low	Non thyroidal illness / Secondary Hypothyroidism	

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Sample Type	: Serum	Report Date	: 17/Apr/2023 03:14PM

Immunology

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Vitamin D (25-OH)	**28.1**	ng/ml	Deficiency:< 20, Insufficiency:20-29, Sufficiency:30 - 100, Toxicity possible:> 100	CMIA

Comment:

- Vitamin D is a fat-soluble steroid prohormone involved in the intestinal absorption of calcium and the regulation of calcium homeostasis.
- Two forms of vitamin D are biologically relevant - vitamin D3 (Cholecalciferol) and vitamin D2 (Ergocalciferol).
- Both vitamins D3 and D2 can be absorbed from food but only an estimated 10-20perc. of vitamin D is supplied through nutritional intake.
- Vitamin D is converted to the active hormone 1,25-(OH)2-vitamin D (Calcitriol) through two hydroxylation reactions. The first hydroxylation converts vitamin D into 25-OH vitamin D and occurs in the liver. The second hydroxylation converts 25-OH vitamin D into the biologically active 1,25-(OH)2-vitamin D and occurs in the kidneys as well as in many other cells of the body.

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Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum	Report Date	: 17/Apr/2023 03:18PM

Immunology

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Vitamin B12	633.0	pg/mL	187 - 883	CMIA

Comment:

- **Vitamin B12** along with **folate** is essential for DNA synthesis and myelin formation.
- **Decreased levels** are seen in anaemia, term pregnancy, vegetarian diet, intrinsic factor deficiency, partial gastrectomy/ileal damage, celiac disease, oral contraceptive use, parasitic infestation, pancreatic deficiency, treated epilepsy, smoking, hemodialysis and advanced age.
- **Increased levels** are seen in renal failure, hepatocelluar disorders, myeloproliferative disorders and at times with excess supplementation of vitamins pills.

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Barcode ID/Order ID	: D2918008 / 7076034	Sample Receive Date	: 17/Apr/2023 11:27AM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Urine	Report Date	: 17/Apr/2023 12:43PM

CLINICAL PATHOLOGY

COMPREHENSIVE SILVER FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Urine Routine & Microscopy				
Colour	PALE YELLOW		Pale Yellow	Visual
Appearance	**TURBID**		Clear	Visual
Specific gravity	>=1.030		1.005 - 1.030	pKa change
pH	<=5.0		5.0 - 8.5	Double Indicator
Glucose	Negative		Negative	GOD-POD
Protein	Negative		Negative	Protein- error principle
Ketones	Negative		Negative	Nitroprusside
Blood	Negative		Negative	Visual
Bilirubin	NEGATIVE		Negative	Diazonium
Urobilinogen	Normal		Normal	Azo Dye
Leucocyte Esterase	**3+**		Negative	Pyrrole
Nitrite	Negative		Negative	Sulphanilamide Diazo
Pus cells	**35-40**		0 - 5/HPF	Microscopy
Red Blood Cells	NIL		Nil	Microscopy
Epithelial cells	2-3		0 - 5/HPF	Microscopy

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